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HISTORY OF PUBLIC HEALTH IN CANADA

A.E. Grauer: The Phantom of Public Health

Anita L. Kozyrskyj

On May 3, 1940, The Rowell-Sirois Commission on Dominion-Provincial Relations presented its report to the Prime Minister of Canada. The report represented an extensive investigation of Canada's federal and provincial social security system.¹ Included was a specialized study on public health prepared by A.E. Grauer—the first comprehensive report to address public health issues in Canada. This paper examines the Grauer report in the context of its historical background, impact on health care policy and contemporary significance.

Historical background

The settlement of the "New World" brought with it the ravages of disease. Health was handled by the individual and his family. Hospitals were administered by religious groups and charities. Temporary local boards of health were established to control epidemics of cholera and typhus. At the time of Confederation in 1867, the main health problems were outbreaks of cholera, typhus and smallpox, for which the only control mechanism was maritime quarantine.² The late 19th century saw radical changes in the knowledge of disease causation, but also increased spread of infection. Provinces passed public health acts, modelled after the UK Public Health Act of 1875, which required municipalities to appoint a permanent local board of health, medical officers of health and sanitary inspectors to deal with communicable

diseases, sanitation and medical assistance to the poor.²

After World War I public health attention extended to maternal and child health services. Provinces established provincial public health departments and a federal department of health was established in 1919.² That same year, the Liberal party passed a resolution for a "system of insurance against unemployment, sickness, dependence in old age, and other disability" to be instituted by the federal government in conjunction with the provinces.³ In 1928 a parliamentary committee investigated the possibilities of social insurance, including health insurance, but federal government opinion was that social security was a provincial responsibility.⁴

During the Depression in the 1930s, the provinces turned to the federal government for help. It is estimated that from 1931 to 1941 the federal government gave out one billion dollars in grants-in-aid to the provinces.² The Depression gave rise to a period of inquiry concerning the adequacy of social services. Physician practices had declined in volume by 37% between 1929 and 1933; indigents flooded the hospitals. In response, the Canadian Medical Association presented a plan for national health insurance in 1934.⁵ Voluntary health insurance plans such as Blue Cross⁵ were developed in the late 1930s when proposed provincial plans were not implemented.⁶

In 1935, the Employment and Social Insurance Act was introduced to provide social security benefits, including health benefits, to the unemployed. The Act was declared *ultra vires* of the British North America Act because social security was considered a provincial jurisdiction.² This was the beginning of a constitutional controversy which exists to this day. The

provinces were reluctant to surrender jurisdiction and the federal government was unwilling to assume obligations without provincial support. To clarify the issue of provincial and federal rights, the Rowell-Sirois Commission was established.⁴

Content of the Grauer Report

An overview of the organization of public health services is provided by Grauer at the beginning of his report on the state of public health in Canada in the late 1930s.⁷ Public health fell under the auspices of the provinces as a "matter of a local and private nature" specified by the British North America Act of 1867. Every province had a department of health which supervised municipal health services. Provincial or local boards of health administered health regulations. Provinces were divided into health districts serviced by physicians, medical officers, public health nurses and sanitary inspectors. The federal Department of Health was responsible for quarantine, marine hospitals, and the health of Aboriginal people. Federal conditional grants were given to provinces to develop health programs for the eradication of diseases, such as venereal disease. Provinces were required to match federal grant contributions and present their plans for the provision of these programs.

Grauer began his evaluation of public health services by noting the dramatic decline in communicable diseases such as diphtheria and typhus, from 1927 to 1936. Vaccination had led to a 90% decrease in deaths from diphtheria in cities like Montreal. However, Grauer pointed out that, despite conditional grants for venereal disease programs, a marked decrease in venereal disease had not occurred. Moreover, tuberculosis was still a serious

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problem, with national statistics of 61 deaths per 100,000 population. The highest death rates were reported in Quebec, where municipal expenditures on prevention and treatment were lowest. Grauer recommended that all provinces assume the full treatment costs for tuberculosis, as Ontario, Alberta and Saskatchewan had done. From 1900 to 1936, Ontario's tuberculosis death rate had fallen from 160 to 36 per 100,000 persons. Rather than increased hospital beds, Grauer argued for more outpatient facilities to manage tuberculosis; British Columbia's province-wide system of clinics had resulted in a 50% reduction of far-advanced cases. In addition, Grauer advocated that provinces administer tuberculosis control programs to combat tuberculosis among Aboriginal people, whose death rate from the disease was 12 times that of the white population.

A drop in infant mortality had also occurred from 1926 to 1936, but at 66 per 1,000 live births, Grauer's statistics showed that Canada's infant mortality was higher than 16 other developed countries. Maternal mortality essentially had remained unchanged at 5.6 per 1,000 live births. Grauer noted that infant mortality was the highest in rural areas, reflecting inadequate services at the time of delivery. Mental health services were also inadequate, with overcrowding of mental facilities and a lack of psychiatry training schools. Grauer proposed preventive mental health measures which included public education, the training of social services personnel in mental health, and community programs for the mentally ill. He anticipated that a comprehensive mental health program would lead to savings from decreased hospital costs, increased productivity and decreased crime.

Local health services were identified as the weakest links in the organization of public health because of their dependence on the wealth of the municipality. Many were understaffed and their personnel unqualified; accessibility to clinics was a problem because of the expense of travel. This left vast areas of the country with few or no health services. Grauer, therefore, recommended a nationwide system of adequately staffed health units. He encouraged provincial health organizations to take a

greater leadership role and to restructure health services, coordinating them with other social services. These recommendations are virtually identical to those outlined in the Hastings' Community Health Centre report thirty years later.⁸

Grauer recommended increased funding for public health programs as an inexpensive means of reducing existing health care expenditures and increasing population welfare. In his view, governments considerably underspent in the area of public health which constituted 15% of provincial and 3% of federal health expenditures, respectively. Grauer proposed that the federal government offer the provinces conditional grants-in-aid for public health programs, which would enable the federal government to provide leadership without infringing upon provincial rights. He also suggested that the control of communicable diseases and the transferability of health services could be more efficiently managed by the federal government.

Impact of the Grauer Report on the recommendations of the Rowell-Sirois Commission

Although the members of the Rowell-Sirois Commission utilized Grauer's data and were "impressed by the inadequacy of health services," unlike Grauer, they concluded that public health administration was a local responsibility. The Commission rejected Grauer's proposals for federal grants-in-aid to the provinces for health services because of potential problems in controlling the costs of provincial administration of these programs.⁹ The Rowell-Sirois report had little influence on future directions of Canadian federalism,¹ but the Commission's concerns over conditional grants might have been justified in light of the inability of the federal government in the 1980s to control health care costs.⁶

Contemporary significance of the Grauer Report

The Grauer report was a milestone in public health in Canada, both in Grauer's application of epidemiologic methods and his recommendations. Despite limitations in public health statistics, Grauer looked at health outcomes to assess the effectiveness

of health care programs. Ahead of his time and certainly in step with our time, he performed economic evaluations of health care programs, determining the "costs of illness" to support arguments for increasing the number of preventive programs in the control of disease. Grauer considered the "costs of illness" from a societal point of view, including the costs of social assistance and decreased productivity.^{7,10} It has been said that if more attention had been paid to epidemiological methods in the planning for health needs, the establishment of Canadian health services wouldn't have been so piecemeal.¹¹

Moreover, Grauer identified problems of accessibility and transferability; both issues were subsequently incorporated as two principles of our Medicare program. He recommended the use of conditional grants, which became the basis of federal-provincial funding of health insurance until 1977.⁶ He identified the economics of home care, now being advocated.⁷ Above all, he advocated health promotion and disease prevention,⁷ themes that are recurrent in our health care history, from the recommendations of Hastings,⁸ to the internationally renowned Lalonde report.¹² It is surprising that Grauer is referred to so infrequently in the literature.

And what has been our track record with respect to public health and health outcomes? In the broader sense, the number of deaths attributable to tuberculosis has decreased dramatically, as have infant and maternal mortality.^{13,14} However, in the 1990s the incidence of tuberculosis in Aboriginal people is nine times that of the Canadian average. In addition, death rates from cardiovascular disease have doubled and those from cancer have tripled from 1926 to 1990.¹³ Cancer and cardiovascular disease have been associated with lifestyle factors, such as smoking, which are amenable to public health education.¹⁵ At 4% of health expenditures, public health still does not appear to be a priority.⁶ In a scramble to control rising health care costs, today's health care reform strategies are aimed at reallocation of resources from the treatment of illness to health promotion and disease prevention.¹⁶ If policy makers had followed Grauer's recommendations, perhaps we would be reaping the benefits

of improved health at a lower cost to our society.

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